KET QUA SINH HOA MAU

Patient:	PO	Species:	Canine	Patient ID:	
Client:		Gender:		Sample No.:	95
Doctor:		Age:	Adult 9Y	Time of analysis:	2026/04/21 08:25

	Item		Current result		Ref. Ranges
Protein	**TP**	↑	**96.4**	g/L	53.1-79.2
Protein	**ALB**		**25.5**	g/L	23.4-40.0
Protein	**GLOB**	↑	**70.8**	g/L	25.4-52.0
Protein	**A/G**		**0.4**		
Liver and gallbladder	**ALT**		**37.9**	U/L	10.1-100.3
Liver and gallbladder	**AST**		**25.3**	U/L	0.0-51.7
Liver and gallbladder	**AST/ALT**		**0.67**		
Liver and gallbladder	**ALP**		**18.5**	U/L	15.5-212.0
Liver and gallbladder	**GGT**		**<2.0**	U/L	0.0-15.9
Liver and gallbladder	**TBIL**		**<1.70**	μmol/L	0.00-15.00
Liver and gallbladder	**TBA**		**2.1**	μmol/L	0.0-30.0
Pancreas	**AMY**		**813.0**	U/L	397.7-1285.1
Kidneys	**BUN**		**6.78**	mmol/L	2.50-9.77
Kidneys	**CREA**		**51.40**	μmol/L	20.00-123.70
Kidneys	**BUN/CREA**		**32.7**		
Cardiovasc./Muscle	**CK**		**116.7**	U/L	66.4-257.5
Cardiovasc./Muscle	**LDH**		**87.3**	U/L	0.0-143.6
Energy metabolism	**GLU**	↑	**7.74**	mmol/L	3.80-7.50
Energy metabolism	**TC**		**6.63**	mmol/L	2.67-8.38
Energy metabolism	**TG**		**0.62**	mmol/L	0.10-1.30
Minerals	**Ca**		**2.54**	mmol/L	2.10-2.97
Minerals	**PHOS**		**1.80**	mmol/L	0.80-2.20
Minerals	**CaxP**		**4.56**	mmol/L^2	
Minerals	**Mg**		**0.56**	mmol/L	0.53-1.06
Electrolytes	**Na+**		**145.5**	mmol/L	138.0-160.0
Electrolytes	**K+**		**4.9**	mmol/L	3.5-5.9
Electrolytes	**Na/K**		**29.7**		
Electrolytes	**Cl-**		**121.5**	mmol/L	102.7-125.0

Operator:

Comprehensive Diagnosis Panel **QC QC OK**

HEM(Hemolysis degree):	0	LIP(Lipemia degree):	0	ICT(Jaundice degree):	0

Report Explan.

TP ↑

Increase is commonly associated with dehydration and increased globulin. Reduction is commonly associated with blood loss, protein-losing enteropathy, and decreased albumin.

The results only applies to this test sample.

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Report Explan.

GLOB	↑	Increase is commonly associated with chronic inflammation and infection, and hyperimmunity, etc. Reduction is commonly associated with insufficient protein intake, anemia, and immunodeficiency.
GLU	↑	Increase is commonly associated with diabetes and hypercorticalismus, etc. Reduction is commonly associated with insulin administration, malnutrition, and insulinoma, etc.

Note: Due to the complexity and individuality of disease diagnosis, the report interpretation is only for your reference. Please consult your doctors for clinical diagnosis results.

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